

Asking your specialist for a letter or your reports

The Alberta Disability System Breakdown | AISH and ADAP | A community-made form. Not a government form.

A request you can send to a specialist who treats you, to support an application for Alberta disability income assistance (AISH or ADAP).

Keep this sentence on the request: This is a formal access request under the Health Information Act.

Today's date

Your full name

Your date of birth

Your phone or email

Specialist's name

Clinic name

Your patient or chart number at this clinic, if you know it

Roughly when you first saw them

What this specialist treats you for

Limit: 110 characters.

Your family doctor's name

Please send it by this date, if you can

I am applying for disability income assistance in Alberta. The application needs a medical report from a doctor. You are one of the doctors who treats me, so what you have found matters to it.

I am asking for one of two things. Either one helps. The second one may be easier for you.

- 1. A short letter about my care with you.** What you diagnosed, what you have treated, what limits you have seen, and what you expect going forward. One or two pages is plenty. The next page sets out what the application weighs, so nothing is wasted.
- 2. Or copies of the consult reports you already sent my family doctor.** If a letter is not possible, these work. They are already written, and my family doctor can attach them to the medical report.

Please send either one to me, or direct to my family doctor, whichever is easier.

If there is a fee, please tell me the amount before the work is done so I can plan for it.

Thank you for your time.

Your signature (type your name, or print and sign)

Date signed

For the clinician

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This page is for the clinician. It is a short guide, not an instruction.

Before anything else

Please write only what your own records and clinical opinion support. Accuracy is the strongest position and the one that holds up on review. An overstated report can be challenged and can cost a patient the case they truly have. Anything below that does not fit this patient should simply be left out.

What the decision turns on

Whether this patient has a severe disability, and how far it affects employment. Alberta now has two programs with two different tests:

AISH requires a severe disability that **permanently prevents employment**.

ADAP is for a severe disability that **substantially impedes employment**.

"Severe disability" means a substantial impairment of mental or physical functioning that is permanent or expected to last at least two years, where all relevant treatment that could lessen it has been completed and no improvement is expected. A diagnosis alone does not establish this. The decision weighs what the condition prevents and whether that will last.

What helps most, within your own area

1. **Diagnoses you have made or confirmed, with onset dates.** Early or longstanding onset supports permanence.
2. **What you have treated, over what period, and how often you see this patient.** A long, regular treating relationship carries weight. Episodic contact is worth saying too, because it frames the scope of your opinion.
3. **The limitations you have observed: what the symptoms prevent, not only what they are.** Sustained concentration, reliable attendance, self-initiation of tasks, lifting, standing, and so on.
4. **Variability, as a range and not an average.** "Severe on flare days, mild between" carries more than a blended "moderate", because the pattern is what gets weighed.
5. **What has been tried and what is exhausted, with rough timeframes and outcomes.** This goes to the second half of the test. If something is available in principle but carries a long wait, that is worth stating.
6. **Prognosis: permanent, episodic, temporary or undetermined.** If episodic, the frequency, length and severity of episodes, and what remains between them.
7. **Whether workplace accommodation would make reliable, sustained work possible.** If in your opinion it would not, saying so and saying why is the single most useful line in the letter.

The simpler alternative, and where this lands

If a letter is not possible, copies of the consult reports already sent to this patient's family doctor serve the same purpose and need nothing new written. A patient may also ask for them as a formal access request under the Health Information Act.

These points feed the Disability Income Assistance Medical Report (DS2444B) at Q5 to Q7 diagnosis and history, Q11 to Q68 degree and level of impairment, Q69 to Q74 treatment and permanence, Q75 prognosis, and Q76 the closing summary. A letter that speaks to them saves a later request for more information.

Notes for you

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These notes are for you, not for the clinic. Keep this page.

Sending it

Send this to the clinic, not only to the doctor. Front desk staff often deal with record requests without the doctor being involved. Keep a copy of what you sent and the date you sent it. That date matters later.

Say it is a formal request

Page 1 already carries this sentence. Leave it there: **This is a formal access request under the Health Information Act.**

Some clinics and Alberta Health Services offer an informal route. It can be quicker. But the review by the Commissioner is attached to the formal route, so going informal can mean giving up your review without being told you did.

If they say no to a letter

They can decline to write something new. Copies of records that already exist are different. A clinic can only refuse those for a few reasons the law lists, and being busy is not one of them.

Your right to copies is section 7 of the Health Information Act. The clinic has 30 days to reply. It can take up to 30 days more, but only for set reasons, and it has to tell you the reason. If it refuses, section 12 says it must give you the reason in writing and name the section of the Act it rests on.

Asking for a review

If the clinic refuses, or does not reply at all, you can ask the Information and Privacy Commissioner to review it. Asking is free. Ask within 60 days of being told their decision. That limit does not apply if they simply never answered you, so being ignored does not cost you the review.

Office of the Information and Privacy Commissioner of Alberta: 780-422-6860, or toll free 1-888-878-4044. Website: oipc.ab.ca

What it can cost

There can be a basic fee of \$25. The clinic does not have to start work until it is paid. Copies are about 25 cents a page.

If the job will cost more, the clinic must send you a written estimate. You then have 20 days to accept it or to ask for fewer records. The clock stops while you decide, and starts again when you agree and pay at least half. The rest is due when you get the records, and anything you overpaid comes back.

You can ask to be let off all or part of the fee. The clinic is allowed to say yes if it thinks that is fair. If it says no, that decision can be reviewed too.

Why the consult reports are worth asking for

A specialist who sees you on a referral is expected to write back to the doctor who referred you. The College of Physicians and Surgeons of Alberta sets 30 days from the first visit. That report covers what they concluded and what they advise next. So a report usually exists already, and you are not asking anyone to write something new.

Your record of this request

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Fill this in as you go and keep it with your copy of the request. If you ever need to ask for a review, these dates are what you will be asked for.

Date you sent this

How you sent it

Date they replied

Day 31 (30 days after you sent it, plus one day)

Fee they quoted, if any

New deadline, if they told you they need more time

Date of their decision

Review deadline (60 days after their decision, if they gave one)

What you were told, and by whom

Limit: 900 characters.

Where this comes from

Health Information Act, sections 7, 10, 11, 12, 15, 73 and 74. Health Information Regulation (AR 70/2001), sections 10 to 13. Both as read on 2 October 2026. The 30-day reporting expectation for consultants is a College of Physicians and Surgeons of Alberta standard of practice (Referral Consultation), not a statute. The AISH and ADAP tests on page 2 are from the Assured Income for the Severely Handicapped Act and its General Regulation (AR 96/2026).